

problems of this condition are to do with conceiving). During your pregnancy, you may find that your symptoms of endometriosis improve or disappear, but it is likely that they will resume when your menstruation begins again after pregnancy and breast-feeding (if you are doing so) are over.

Uterus infection

» What is it?

Chorioamnionitis is an infection in pregnancy of the membranes surrounding the fetus. This is associated with prolonged rupture of the membranes (if there is a long period of time between your membranes rupturing and birth) and can affect mom and baby, so labor needs to be induced and you'll be given antibiotics. Chorioamnionitis makes a uterus infection after birth, known as endometritis, more likely. Endometritis is also more likely if you have had a C-section or a retained placenta. The infection causes pain in the lower abdomen, and you may have a high temperature. Antibiotics usually clear the infection.

» I've had an infection in my uterus in the past and may have scarring. Will this make it harder for me to carry a baby to term?

Uterine infection can cause scar tissue to develop in the uterine lining (endometrium). This, in turn, can make it harder for a fertilized egg to implant, and if it does implant, this may be less secure, increasing the risk of miscarriage. You're also at increased risk of placenta previa (see right) and placental abruption (see below). Inform your doctor of your medical history. You will be given regular checkups to monitor the progress of your pregnancy.

Cervical insufficiency

» What is it?

Cervical insufficiency is a cervix that is not rigid enough to safely hold in the membranes surrounding your baby. The cervix could open too early, in the second trimester.

» How is this diagnosed, and what does it mean?

Cervical insufficiency may be the result of a previous pregnancy that stretched the opening of the cervix too far, or of previous cervical surgery. If undiagnosed, you are at increased risk of late miscarriage, when your baby is heavy and the membranes are forced out of the cervix and rupture. However, most cases are picked up during prenatal checkups, before becoming problematic. Your doctor may send you for an ultrasound to look at your cervix (unless you've had a previous cervical insufficiency, in which case an ultrasound isn't needed) and put a temporary stitch—a cerclage—into the cervix between weeks 12 and 16 to hold things in place. The stitch is removed before labor. You may be given medicine if the cervix starts to open up and shorten.

Placenta previa

» What is placenta previa?

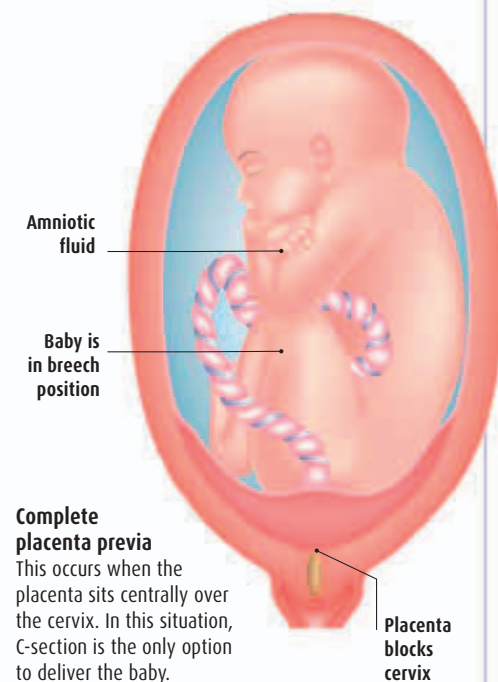
More commonly known as “low-lying placenta,” placenta previa occurs when the placenta's position partially or completely covers your cervix—and thus blocks your baby's exit to the birth canal. A diagnosis of placenta previa is not certain until late pregnancy because the placenta can move away from the internal opening of the cervix (os) as the uterus grows and stretches. The sonographer will check the position of your placenta at your second ultrasound (the anatomy scan). If your baby's placenta is noted to be low-lying, you will have another ultrasound at between 32 and 34 weeks to see whether the placenta has shifted upward. Around 90 percent of low-lying placentas seen before 28 weeks will move to a better position by the follow-up ultrasound. (This happens in around 50 percent of cases if you have had a C-section in the past.) Placenta previa can be the result of fibroids or scar tissue preventing the placenta from attaching in a good position, or it might happen by chance.

» What impact does a diagnosis of placenta previa have on my pregnancy?

Most cases of placenta previa require a C-section because a vaginal delivery isn't possible if the placenta is $\frac{3}{4}$ in (2 cm) from the edge of the cervix or closer. Your doctor will talk to you about the possibility of an elective (planned) C-section in order to deliver your baby safely. There could be some extra fetal monitoring, too.

In addition to blocking the cervix, placenta previa can cause painless, bright red bleeding. If you notice this

kind of bleeding, let your doctor know immediately. Heavy bleeding can put both you and your baby at risk, but most of the time it can be treated before there is any danger. In severe cases, you might need a blood transfusion. Your obstetrician might advise that you abstain from intercourse to prevent the risk of bleeding, or even advise bed rest or a hospital stay. If there is profuse bleeding or risk of premature birth, your doctor could recommend an emergency C-section.



Although placenta previa often makes a **cesarean section necessary**, the vast majority of women deliver **healthy babies** and recover well from their surgery.

Placental abruption

» What is it?

Placental abruption, when part or all of the placenta comes away from the wall of the uterus, is a serious condition that significantly impacts your baby's ability to receive nutrients and oxygen. Rarely, the entire placenta may become detached. The first signs that it might be happening are abdominal pain and bleeding. If you experience any spotting during your pregnancy, call your doctor. It is a potentially life-threatening condition, so immediate attention is imperative.

» My friend's baby was stillborn following a placental abruption. What can I do to stop this from happening to me?

Though you can't do anything specifically to stop a placental abruption, it's possible to carry a baby to term as long as the abruption is only mild—meaning that most of the placenta remains attached to the uterus (enough to nourish your baby). Severe or complete abruption is not treatable and is more likely to result in premature birth, or stillbirth. It is also dangerous for you, since it can result in significant blood loss. If you smoke or take drugs—stop. These habits are associated with an increased risk of placental abruption. Preeclampsia and a small baby also increase the risk.